INFORMATION TO KEEP WITH YOU

My complete treatment handbook

TRAIN-2 with Phesgo

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00790
Version 3

Six 21-day cycles before surgery: Phesgo and carboplatin on Day 1; paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Surgery and pathology determine postoperative HER2 therapy.

Treatment stages

01 **Chemotherapy with Phesgo**
21-day cycle

02 **Surgery / pathology and postoperative plan**
Separate phase

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for new breathlessness, swelling, severe diarrhoea, inability to drink or sudden illness. Tell staff immediately about rash, chest tightness or breathing difficulty during the injection. Fever, shivers, unusual bleeding or worsening numbness during chemotherapy also need prompt advice.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Two antibodies in one injection with chemotherapy

Shrink the cancer before surgery and contribute to reducing recurrence risk as part of a complete early-breast plan.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next next dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

PLATINUM CHEMOTHERAPY

Carboplatin



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) inside cells

Carboplatin creates links in DNA, making it harder for cells to copy and repair their genetic instructions. Its dose depends on kidney function as well as the intended regimen.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Keep blood-test appointments. Fever, unusual bleeding, repeated vomiting or infusion symptoms need prompt advice.

HER2-TARGETED ANTIBODY

Pertuzumab

Part of the combined Phesgo injection



Blocks HER2 pairing with other receptors

Pertuzumab binds a different area of HER2 from trastuzumab. It limits receptor pairing, so the two antibodies act together at different points on the same growth pathway.

HER2 pairing → Pertuzumab blocks pairing → Growth signals reduced

What to watch for

Keep echo appointments. Report diarrhoea, new breathlessness, swelling, palpitations or an injection reaction.

HER2-TARGETED ANTIBODY

Trastuzumab

Part of the combined Phesgo injection



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

The aim is to treat higher-risk HER2-positive breast cancer before surgery and reduce the chance of it returning. Paclitaxel and carboplatin are chemotherapy medicines; Phesgo contains two medicines that target HER2.

How is the treatment organised?

The NCCP plan uses six cycles without an anthracycline: Phesgo and carboplatin on Day 1, with paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Your team considers the cancer features, general health, nerve symptoms and visit pattern. Surgery and pathology then guide the next treatment.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

The aim and the whole plan

Who this is for

HER2-positive early, locally advanced or inflammatory breast cancer at high risk of recurrence, receiving neoadjuvant treatment.

Schedule

Six 21-day cycles before surgery: Phesgo and carboplatin on Day 1; paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Surgery and pathology determine postoperative HER2 therapy.

After this phase

After surgery, your team reviews the pathology and chooses the next HER2 medicine. The usual plan aims to complete about one year of HER2 treatment from the first dose. Remaining invasive cancer can change the medicine offered. The selected treatment has its own blood tests and supportive medicines.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

How the treatment phases connect

Treatment stages

01 **Chemotherapy with Phesgo**
21-day cycle

02 **Surgery / pathology and postoperative plan**
Separate phase

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE

Chemotherapy with Phesgo

6 cycles

Six 21-day cycles before surgery: Phesgo and carboplatin on Day 1; paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Surgery and pathology determine postoperative HER2 therapy.

SEPARATE PHASE

Surgery / pathology and postoperative plan

Individual plan

After surgery, your team reviews the pathology and chooses the next HER2 medicine. The usual plan aims to complete about one year of HER2 treatment from the first dose. Remaining invasive cancer can change the medicine offered. The selected treatment has its own blood tests and supportive medicines.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal durations. The first routine 12-week echo falls during the six-cycle preoperative course, around cycle 5 if uninterrupted. Count from first HER2 treatment; a delay, surgery or a change of medicine does not restart the clock. The selected postoperative regimen determines later tests.

Check	Before starting	Cycles 1–4	Around cycle 5 / week 12	Cycle 6 / surgery	Postoperative plan
Blood tests	full blood count (FBC), renal and liver profile; verify unindexed GFR for carboplatin	FBC, renal and liver profile before every cycle. Review required counts before Day 8 dosing under the local pre-dose pathway.	Continue pre-cycle bloods	Before cycle 6; preoperative tests as directed	Follow the selected postoperative medicine; do not carry forward taxane-day checks
Echo / multigated acquisition (heart pumping) scan (MUGA)	Before the first HER2 treatment	Earlier if symptoms or concern	Echo due at about 12 weeks (cycle 5 if on time)	Review any due echo; earlier if symptoms	Continue on the selected HER2 plan; surgery does not restart the clock
electrocardiogram (ECG) / cardiac markers	ESC: baseline ECG and cardiovascular risk review; baseline troponin/BNP if they will be used for monitoring	Repeat when symptoms, risk or the cardiac plan indicate	ECG does not replace echo	Markers are risk-based, not automatic tumour-marker tests	Follow up abnormal results
Scans / cancer markers	Document breast/nodal disease and staging as indicated	Clinical response; earlier imaging for concern	multidisciplinary team (MDT)-selected response imaging; not automatically tied to the week-12 echo	Reassess before surgery	Breast surveillance; symptom-directed other imaging

Check	Before starting	Cycles 1–4	Around cycle 5 / week 12	Cycle 6 / surgery	Postoperative plan
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	Assess endocrine indication and fracture risk	No routine cycle-by-cycle DEXA	Plan baseline DEXA if aromatase inhibitor (AI) / ovarian suppression indicated	Repeat according to risk, results and endocrine therapy	Continue the agreed bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00790 v3 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Phesgo product information

THE WHOLE TREATMENT JOURNEY

TRAIN-2 with Phesgo: the whole pathway

HER2-positive neoadjuvant breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Baseline	Neoadjuvant cycles 1–6	Surgery / pathology	Postoperative treatment	Follow-up
Cancer medicines NCCP 00790	Confirm HER2 status, indication and baseline checks	Phesgo + carboplatin Day 1; paclitaxel Days 1, 8; six cycles	Surgery and pathology select adjuvant medicines	Selected adjuvant HER2 course, aiming for one year overall	Review outcomes and next plan
Surgery EARLY	Plan after neoadjuvant therapy	Systemic treatment first	Lumpectomy / mastectomy after recovery	Pathology guides the next medicines	Local follow-up
Radiotherapy EARLY	Assess indication	Usually after chemotherapy and surgery	Plan after surgery / recovery	May overlap HER2-only treatment if agreed	Review late effects

Care / check	Baseline	Neoadjuvant cycles 1–6	Surgery / pathology	Postoperative treatment	Follow-up
Blood tests NCCP 00790	Baseline FBC, renal / liver	FBC, renal and liver profile before every cycle. Review required counts before Day 8 dosing under the local pre-dose pathway.	Check before changing phase	Use the blood-test schedule for the separately selected postoperative medicine	Active medicines / abnormal results determine follow-up
Echo / heart tests NCCP 00790 · ESC · SMPC	Baseline echo / MUGA; ECG and cardiovascular risk review	Echo every 12 weeks: first around cycle 5; earlier for symptoms	Review any due cardiac test; surgery does not restart the clock	Continue cardiac checks for the selected HER2 medicine	Agree cardiac follow-up when treatment ends
Imaging / markers EARLY · MARKERS	Baseline breast / nodal assessment; staging as indicated	Clinical response and MDT-selected imaging during treatment	Reassess before surgery; pathology informs the next plan	Mammography of remaining breast tissue under the surveillance plan	Other imaging for symptoms / clinical concern; no routine tumour-marker screening
Endocrine / DEXA BONE · EARLY	Check hormone receptors and fracture risk	Discuss the later plan	Baseline DEXA when indicated	Endocrine therapy may overlap HER2 / radiotherapy (RT) if agreed	Bone and endocrine follow-up

What each check is for

1

Blood tests

FBC, renal and liver profile before every cycle. Review required counts before Day 8 dosing under the local pre-dose pathway.

Check treatment safety.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

2

Echo / MUGA

Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Monitor heart pumping function.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

3

Cancer imaging

Baseline breast/nodal assessment and staging as indicated; assess response during neoadjuvant treatment and before surgery using the MDT-selected imaging. After local treatment, arrange mammography of remaining breast tissue; routine computed tomography (CT)/ positron emission tomography (PET) or tumour markers are not asymptomatic early-breast recurrence screening.

Assess disease response or surveillance.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

4

Bone health

If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Reduce fracture risk when endocrine therapy affects bone.

Ask / act: The treating team records the booking owner, due window, result reviewer and action if overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

Phesgo is one combined injection of pertuzumab and trastuzumab under the skin of the thigh. It is not two separate injections and is never given into a vein.

STEP 1

Before the visit

Confirm blood results, cardiac checks, active phase, previous dual HER2 dose and the consent discussion.

STEP 2

The thigh injection

Loading/reloading 1200 mg/600 mg over 8 minutes with 30 minutes observation; maintenance 600 mg/600 mg over 5 minutes with 15 minutes observation. Complete the observation before chemotherapy. Alternate thighs; avoid sore, bruised or damaged skin.

STEP 3

Chemotherapy and access

After Phesgo observation, chemotherapy is given through a cannula, peripherally inserted central catheter (PICC) or port. The team explains access insertion and care. No take-home infusion pump is routinely used.

STEP 4

Leaving the unit

Confirm supportive medicines, next visits and due tests. Injection time is shorter than the whole appointment; preparation, assessment and observation still take time.

Switching / a missed appointment

The team checks the last dual HER2 dose, including intravenous (IV) treatment: a gap of less than 6 weeks uses the maintenance dose; 6 weeks or longer needs reloading. A first Phesgo visit after IV treatment does not automatically require a loading dose.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Scans, heart checks and bloods

Agree who books, reviews and acts on each result.

Blood tests

FBC, renal and liver profile before every cycle. Review required counts before Day 8 dosing under the local pre-dose pathway.

Heart ultrasound

Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Longer-term cardiac plan

After treatment ends, agree cardiac follow-up from symptoms, risk, prior anthracyclines and the next medicine; use the active medicine's product information and cardio-oncology plan.

Cancer imaging and markers

Baseline breast/nodal assessment and staging as indicated; assess response during neoadjuvant treatment and before surgery using the MDT-selected imaging. After local treatment, arrange mammography of remaining breast tissue; routine CT/PET or tumour markers are not asymptomatic early-breast recurrence screening.

When the treatment phase changes

The first 12-week echo is during preoperative treatment, around cycle 5 if on time. After surgery, use the monitoring and supportive medicines for the selected postoperative regimen; the earlier chemotherapy-day blood checks do not automatically continue.

Surgery, radiotherapy and hormone treatment

Other treatment depends on your cancer and personal plan.

How surgery and radiation fit

Systemic neoadjuvant treatment comes before surgery. Lumpectomy versus mastectomy is agreed with the surgeon. Radiotherapy, if indicated, follows surgery/recovery and is coordinated with postoperative systemic treatment; it may overlap HER2-only treatment when both teams agree.

Endocrine treatment and bone health

If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Receptor and genomic tests

Hormone-receptor status can affect endocrine treatment. Oncotype-type chemotherapy triage is not applied to these HER2-positive regimens; treatment selection requires the appropriate indication-specific MDT assessment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Chemotherapy with Phesgo

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Phesgo · Day 1	 **Day 1** One subcutaneous (SC) injection
Paclitaxel · Day 1	 **Day 1** intravenous (IV) + carboplatin
Paclitaxel · Day 8	 **Day 8** IV
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–21** Fever or sudden illness: call
Nerves / skin / nails	 **Days 1–21** Can accumulate across doses
Diarrhoea	 **Days 1–21** Report early; keep drinking
Injection reactions	 **Days 1–21** Report during or after visits
Heart / breathing symptoms	 **Days 1–21** Report symptoms on any day

No dates to enter: Use this picture to understand the sequence. Your team provides your personal appointments.

Changing phase: After surgery, your team reviews the pathology and chooses the next HER2 medicine. The usual plan aims to complete about one year of HER2 treatment from the first dose. Remaining invasive cancer can change the medicine offered. The selected treatment has its own blood tests and supportive medicines.

Echo clock: Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

> **Why these bands stay broad:** Treatment days show the planned pattern. Symptom bands are broad possibilities, not measured risk curves or a prediction of when you will feel unwell. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00790 v3 · official regimen ↗

> **There are no guaranteed "safe days".** Serious symptoms need assessment on any day, even if the chart shows a usual recovery period.

Between-treatment notes

Day 1–21: A reference cycle, without dates to enter

Six 21-day cycles before surgery: Phesgo and carboplatin on Day 1; paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Surgery and pathology determine postoperative HER2 therapy. Your team books actual appointments and reviews results. Report urgent symptoms on any day.

Support around the medicines

1 Antiemetic plan

Carboplatin Day 1 uses its high-risk antiemetic plan. Day 8 paclitaxel uses a separate low-risk plan.

2 Premedicines

First paclitaxel dose: steroid, antihistamine and H2 blocker. Later doses: steroid is not routine unless the consultant adds it; the other premedicines follow the reviewed plan.

3 Appointments and results

Echo before treatment and every 12 weeks (around cycle 5 if on time), including maintenance. Bloods before each cycle. Ask who books and reviews each test; report a missing appointment.

The injection and your visit

Injection and observation

Questions worth asking

Will I need CT scans, markers or DEXA?

Baseline breast/nodal assessment and staging as indicated; assess response during neoadjuvant treatment and before surgery using the MDT-selected imaging. After local treatment, arrange mammography of remaining breast tissue; routine CT/PET or tumour markers are not asymptomatic early-breast recurrence screening. If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Why was this treatment chosen for me?

The aim is to treat higher-risk HER2-positive breast cancer before surgery and reduce the chance of it returning. Paclitaxel and carboplatin are chemotherapy medicines; Phesgo contains two medicines that target HER2. The NCCP plan uses six cycles without an anthracycline: Phesgo and carboplatin on Day 1, with paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Your team considers the cancer features, general health, nerve symptoms and visit pattern. Surgery and pathology then guide the next treatment.

Try explaining the plan back: Which medicines are active now, when is your next echo or scan due, and who will review it?

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Carboplatin

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Allergic reaction, sometimes after previous exposure
- Kidney or electrolyte changes

Pertuzumab

- Diarrhoea and dehydration
- Injection / allergic reaction
- Heart pumping-function reduction
- Rash and fatigue

Trastuzumab

- Heart pumping-function reduction / heart failure
- Injection or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin combination

Active medicines: Paclitaxel + Carboplatin + Pertuzumab + Trastuzumab

Emetogenic risk: High · carboplatin AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 Reconcile chemotherapy premedication steroids
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Individual contraindications, renal function and duration review
Dexamethasone	8 mg orally daily	Days 2–4 Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before paclitaxel.
- Day 8 and thereafter: no routine dexamethasone; chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before. Consultant may add steroid for risk/prior reaction or omit famotidine if appropriate; monitor closely.
- Rechallenge steroid changes require consultant guidance; consider analgesia for taxane aches and granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk.
- Day 1 antiemetic steroid is reconciled with paclitaxel premedication; do not add two steroid courses.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · paclitaxel only

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No carboplatin or Phesgo on Day 8; no planned Day 15 paclitaxel.
- Day 8 and thereafter: no routine dexamethasone; chlorphenamine 10 mg intravenous (IV) and famotidine 20 mg IV 30 minutes before. Consultant may add steroid for risk/prior reaction or omit famotidine if appropriate; monitor closely.
- Rechallenge steroid changes require consultant guidance; consider analgesia for taxane aches and granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Postoperative HER2-only phase · confirm selected regimen

Active medicines: Selected postoperative HER2 medicine; separate source regimen

Emetogenic risk: Confirm the selected postoperative regimen

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Open the separately selected postoperative regimen before issuing supportive medicines. Its emetic risk, premedication and tests apply.
- Phesgo or trastuzumab alone generally needs no routine antiemetic prophylaxis. Do not apply this statement automatically to an antibody-drug conjugate.
- Pathology, including residual invasive disease, informs the next HER2 medicine and remaining treatment course.
- Continue cardiac monitoring on the selected treatment plan; surgery does not restart the clock.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

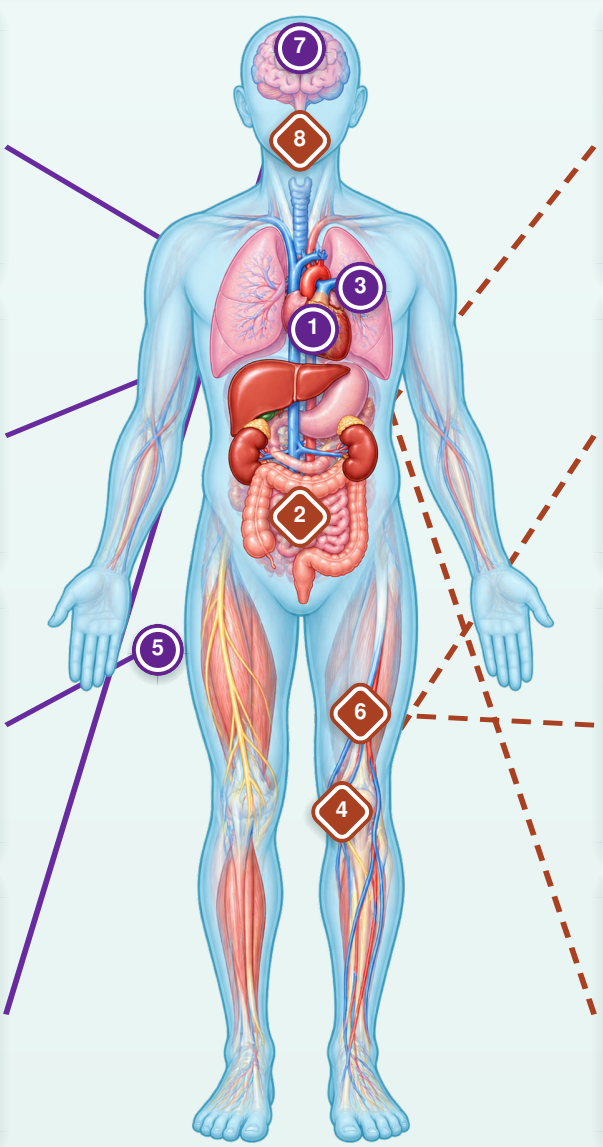
TRAIN-2 with Phesgo

Six 21-day cycles before surgery: Phesgo and carboplatin on Day 1; paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Surgery and pathology determine postoperative HER2 therapy.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Heart / breathing

Phesgo

Keep echoes; report breathlessness, swelling or reduced exercise tolerance.

2 Diarrhoea

Especially pertuzumab

Report watery stools early, particularly with poor intake or dizziness.

3 Injection reaction

Phesgo

Tell staff immediately about rash, chest tightness or breathing difficulty.

4 Thigh injection site

Phesgo

Report persistent or spreading redness, swelling or pain.

5 Hands / feet

Paclitaxel

Report numbness or tingling before the next dose.

6 Blood / infection

Chemotherapy

Fever, shivers or suddenly unwell: call immediately.

7 Hair / scalp

Chemotherapy

Hair loss is common; ask about scalp cooling where suitable.

8 Mouth / appetite

Chemotherapy

Painful ulcers or reduced drinking need advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The body map follows the medicines active in this phase.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Current chemotherapy phase · reusable record · copy this sheet for another cycle of the same phase.

Paclitaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Muscle / joint pain	□ None	□ Mild aches; normal activity	□ Pain limits tasks / sleep: call	□ Pain limits self-care: urgent review	□ Sudden severe deterioration: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999

Carboplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999

Pertuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Trastuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

TRAIN-2 with Phesgo

NCCP 00790 v3 · 21-day cycle

Six 21-day cycles before surgery: Phesgo and carboplatin on Day 1; paclitaxel on Days 1 and 8. There is no planned Day 15 paclitaxel. Surgery and pathology determine postoperative HER2 therapy.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00790/ · no patient information.

Draft for review · SACTCheck v0.84.15

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00790 v3 · official regimen**
   https://healthservice.hse.ie/documents/6442/790_v3_Phesgo_PACLitaxel_CARBOplatin.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **EMA Phesgo product information**
   https://www.ema.europa.eu/en/documents/product-information/phesgo-epar-product-information_en.pdf
7. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
10. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
11. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
12. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf